Chao Phraya Medical Center
1 Arun Amarin Road, Bangkok Noi, Bangkok 10700, Thailand

Form MR-07 / Rev. 3

# DISCHARGE SUMMARY

| | | | |
|---|---|---|---|
| Patient name | **Mrs. Somsri Chaiyaporn** | HN | HN 58-203117 |
| Date of birth | 14/08/1961 | Sex | Female |
| Ward | Medical Ward 7B, bed 12 | | |
| Admission date | 02/01/2024 | Discharge date | 09/01/2024 |

## DIAGNOSIS

**Primary diagnosis:** Community-acquired pneumonia, right lower lobe

**Secondary diagnoses:**

- Type 2 diabetes mellitus
- Essential hypertension

## PROCEDURES PERFORMED

- Chest X-ray
- Bronchoscopy with bronchoalveolar lavage

## HOSPITAL COURSE

The patient presented with a 4-day history of productive cough, fever up to 39.2 C and right-sided pleuritic chest pain. Chest X-ray on admission showed right lower lobe consolidation. Admission blood glucose was 286 mg/dL.

Intravenous ceftriaxone and oral azithromycin were started. Because of slow clinical response by day 3, bronchoscopy with bronchoalveolar lavage was performed; cultures grew Streptococcus pneumoniae sensitive to the current regimen.

Fever settled on day 4 and oxygen was weaned to room air on day 5. Insulin was used during admission and the patient was switched back to oral agents before discharge. Blood pressure remained controlled on her usual medication.

## MEDICATIONS ON DISCHARGE

- Amoxicillin/clavulanate 875/125 mg, 1 tablet twice daily for 5 more days
- Metformin 1000 mg, 1 tablet twice daily with meals
- Amlodipine 5 mg, 1 tablet once daily

## DISCHARGE INSTRUCTIONS

Complete the full course of oral antibiotics. Check blood glucose twice daily and keep a written record. Take blood pressure medication as before. Return to the emergency department immediately if fever above 38.5 C, shortness of breath or chest pain recurs.

## FOLLOW-UP

Respiratory clinic in 2 weeks with repeat chest X-ray. Diabetes clinic in 4 weeks with HbA1c.

**Attending physician**
(Signature)

Dr. Pichit Rattanakorn